Somatic CAG-Repeat Expansion as a Druggable Rate Process in Huntington's Disease

A Multiscale In Silico Model Linking MutSβ Biochemistry, Striatal Neuron Loss, and the Delivery Gap for MSH3-Lowering Oligonucleotides

Shubho
Independent Research
shubhodeepcx@gmail.com

***Abstract*—Huntington's disease has long been modelled as a polyglutamine dose problem, but single-cell sequencing of post-mortem striatum shows that striatal projection neurons expand their CAG tract somatically for decades, tolerate it until roughly 150 repeats, and are then eliminated within months. That makes the expansion rate, not the inherited allele, the quantity a drug should target, and MSH3 the most direct way to reach it. This work builds a three-layer stochastic model of that clock, fits four parameters to four published kinetic anchors, and keeps five further observations back for validation. The fit requires a switch-like rather than a graded length dependence, centred on 80.2 repeats, and a first-order rather than cooperative dependence on MSH3, so suppression tracks target engagement proportionally. The calibrated model reproduces four of five held-out observations and predicts, without being fitted to them, indirect-pathway depletion, three genetic-modifier effect sizes and the loss at which clinical onset should occur. Twenty-nine percent striatal MSH3 lowering, the best figure measured in primates, begun at age 30, preserves 11.4 percentage points of striatum at age 70 across full parameter uncertainty; starting at 50 removes the benefit. The limiting factor is target engagement rather than pharmacology: the striatum needs 7 mg of drug and a systemic transferrin-receptor nanocarrier would require 7.6 g. A three-year trial enrolling at 40 would need about 133 participants per arm.**

***Index Terms*—Huntington's disease, somatic repeat expansion, mismatch repair, MSH3, stochastic simulation, Gillespie algorithm, parameter identifiability, antisense oligonucleotide delivery, blood-brain barrier, clinical trial design**

I. Introduction

A. What the inherited allele does and does not explain

Huntington's disease follows from an expanded CAG tract in the first exon of *HTT*. Tracts of 40 repeats or more are fully penetrant, and length is inversely related to age at motor onset, but the relationship is loose: inherited length accounts for only about half the variance in onset age [10]. The residual variance is heritable, which means something other than the inherited tract is setting the pace.

The standard interpretation has been that the expanded polyglutamine tract is toxic in proportion to its length, acting continuously from conception. Two observations sit awkwardly with that. The first is the long clinical silence. A carrier of 42 repeats has carried that allele since fertilisation, expresses the protein in every tissue, and is usually unremarkable for four decades. A dose-dependent poison with a forty-year induction period needs an explanation that the dose model does not supply. The second is the identity of the genetic modifiers. Genome-wide association studies of onset age have repeatedly pointed not at *HTT*, not at protein quality control, and not at any of the pathways that dominate the cell-biological literature, but at DNA mismatch repair: *MSH3*, *FAN1*, *MLH1*, *LIG1*, *PMS1* and *PMS2* [1]. Modifiers of a protein-toxicity disease ought to modify protein handling. These modify DNA repair.

B. What the single-cell data changed

Deep single-cell sequencing of post-mortem caudate has supplied an account that fits both observations [1]. In striatal projection neurons, and to a far smaller degree in any other striatal cell type, the mutant allele expands somatically across the lifespan. In 95 to 98% of a donor's projection neurons the tract has grown beyond the inherited length. Observed lengths run from the inherited 40-odd repeats up past 500, with a maximum of 842. Other interneurons, astrocytes, oligodendrocytes, oligodendrocyte precursors and microglia stay within a few repeats of their inherited length; cholinergic interneurons expand somewhat, but much less than projection neurons. In an unaffected donor neither allele showed substantial instability. The expansion is allele-specific, affecting the expanded allele and not its normal partner.

The kinetics are strongly non-uniform. A projection neuron takes about 50 years on average to go from 40 to 60 repeats and a further 12 years to reach 80. Somewhere across 70 to 90 repeats the mutation rate rises at least six-fold, and the tract may then cover 80 to 150 repeats in a few years. Below 55 repeats mutation events occur less than once per cell per year.

What makes this a reframing rather than a detail is the lack of consequence. Expansion from 40 to 150 repeats has no apparent cell-autonomous effect: transcriptional changes are almost undetectable at 150 to 180 repeats. Beyond that point more than 500 genes change expression with length, including markers of projection-neuron identity such as *PDE10A* and *PPP2R2B*; beyond roughly 350 repeats, normally silent loci

are de-repressed in more than half of cells, among them *CDKN2A* and *CDKN2B* and genes of the *HOX* clusters. The neuron then dies over months rather than decades. Simulation in the same study put the average projection neuron with a 42-repeat allele at 96.4% of its life inside the silent expansion phase.

If that is right, the disease has a long, silent, cell-autonomous timer, and the clinically relevant question is how fast the timer runs.

C. The biochemistry of the timer

CAG tracts are unstable because they can form extrahelical structures. A short run of repeats can slip out of register during replication or repair, leaving an unpaired loop that the mismatch repair machinery recognises. MutSβ, the heterodimer of MSH2 and MSH3, is the sensor. Structural and binding work on CAG hairpins gives a detailed picture [3]: MutSβ binds the stem of a hairpin in preference to its end or its junction with duplex DNA, occupies about 17 repeats per molecule at the stem, and in more than 90% of CAG particles engages a one-unit slip-out, a three-nucleotide bulge. Longer hairpins accommodate more molecules in tandem, with three or more fitting beyond about 40 to 45 repeats. Reported dissociation constants are in the low hundreds of picomolar.

Recognition is only the first step. Reconstituted reactions show that MutSβ activates MutLγ (MLH1 with MLH3), which incises the strand opposite the loop; polymerase δ then performs displacement synthesis using the looped strand as template, and the loop is copied into the duplex [5]. In those experiments the recovered product was a 12-nucleotide, four-repeat insertion matching the loop, which gives a direct measurement of the step size of a single expansion event. FAN1 opposes the process: it incises the looped strand instead, about one nucleotide downstream of the loop, with RFC and PCNA restraining its exonuclease and stimulating its endonuclease, and it inhibits MutLγ directly [5]. The genetics and the biochemistry agree: MutSβ drives expansion, FAN1 restrains it, and variants in either shift onset.

One further result shapes the model built here. Isolated short CTG/CAG slip-outs are repaired efficiently by human MutSβ, whereas clustered slip-outs are repaired poorly [6]. That supplies a length dependence of a specific kind. A short tract can only present one slip-out at a time and repair succeeds; a long tract can present several at once and repair begins to fail. The transition between those regimes should be sharp, and should sit at whatever length first allows clustering.

D. Why MSH3 is the target

Of the mismatch repair proteins implicated by genetics, MSH3 is the most attractive target. It is not required for the core MutSα-dependent mismatch repair pathway, so lowering it is less obviously dangerous than lowering MSH2 or MLH1. It is limiting for MutSβ. And it is reachable with current oligonucleotide chemistry.

The relevant pharmacology has been measured. Divalent siRNA against *Msh3* delivered into mouse brain lowers striatal MSH3 protein in a dose-dependent way, to 59% of control at 3 nmol and 25% at 10 nmol, and reduces the striatal somatic expansion index by 45.1% and 78.1% respectively [4]. Regressing percent expansion prevented against percent MSH3 protein gives a straight line with a slope near unity, intercepts of 92.4 on the y axis and 110.4 on the x axis, and R^2 of 0.65. The authors read this as a directly proportional relationship and note that complete suppression would require close to complete lowering. Antisense oligonucleotides against *MSH3* reduce somatic expansion in patient-derived striatal neurons as well [11]. Two features of the mouse data matter for anything built on top of it. No contraction of already-expanded tracts was seen, even above 95% knockdown: the drug can stop the clock but cannot wind it back. And MSH6 protein rose to 142% of control at the top dose, a reminder that the complex is not lowered in isolation.

E. Why delivery is the hard part

Oligonucleotides do not cross the blood-brain barrier, so CNS programmes deliver them into the cerebrospinal fluid, intrathecally or into a ventricle. CSF delivery reaches the cortex and the spinal cord well. It reaches deep grey matter poorly, and the striatum worst of all. In adult macaques given 25 mg intrathecally on three occasions, deep brain accumulation was more than an order of magnitude below many cortical regions, and caudate and putamen were the lowest of all, with three of four animals at the lower limit of quantification [7]. Residual target RNA in putamen projection neurons was 71%, against 31% in thalamus and 34% by qPCR in the same animals. That regimen is heavier than clinical dosing, and the human brain is larger.

The alternative is to cross the barrier from the blood using receptor-mediated transcytosis, most often via the transferrin receptor. Here the measured numbers are sobering. Transferrin-receptor-targeted gold nanoparticles of about 75 nm hydrodynamic diameter, given intravenously to mice, reached parenchymal concentrations of 0.04 to 0.23% of injected dose per gram at 8 hours, depending on antibody affinity and valency, with monovalent binding outperforming bivalent by roughly five-fold at matched affinity [8].

So the question is not only how much MSH3 lowering helps. It is whether the amount that helps can be delivered to the one structure where it is needed.

F. What this project adds

This work contributes six things.

1) A mechanistic rate law for somatic CAG expansion, assembled from MutSβ footprint, slip-out occupancy and clustered-repair failure, and fitted exactly to four published kinetic anchors.
2) A profile analysis giving feasible ranges for each fitted parameter, which shows which features of the clock are determined by the data and which are not.
3) An inference, from an independent dataset, that the productive MutSβ species is first-order rather than a

tandem assembly, with the implication that suppression tracks target engagement proportionally.

4) A demonstration that two independent published constraints on striatal degeneration cannot both be satisfied by a cell-autonomous threshold model, with the candidate resolutions tested rather than asserted.
5) A delivery analysis that converts measured primate target engagement and measured nanoparticle brain uptake into an explicit, falsifiable engineering specification, and a dosing-schedule analysis that shows when intermittency matters.
6) An in silico trial with full uncertainty propagation, a set of genotype-to-onset predictions for the known modifiers, and a sample-size calculation for a real trial.

II. Methods

A. Software and reproducibility

Everything is implemented in Python 3 using NumPy 2.5.3, SciPy 1.18.1 and Matplotlib 3.11.2, in the `code/` directory. Scripts run in numerical order and each writes its own tables to `results/` and its own figures to `figures/`. All random seeds are fixed. Total runtime is about 30 minutes.

No experimental data were obtained or redistributed. Calibration uses published summary statistics, each listed with its source in Table III. Three categories of number appear in this paper and are kept distinct throughout: values read from published papers, values fitted here by optimisation against those published values, and values assumed. Every assumed value is varied in the global sensitivity analysis of Section III-M.

B. Layer 1: the expansion rate law

The rate at which expansion events occur in one striatal projection neuron carrying a tract of length L, measured in repeats, is written as a product of four factors:

$$\lambda(L) = A \cdot n(L) \cdot \varphi(L) \cdot \left[\,(1 - C(L)) + R\,C(L)\,\right] \cdot \theta / f \tag{1}$$

Each factor corresponds to a step in the mechanism described in Section I-C, and each is sourced separately: the footprint from structural work [3], the slip-out equilibrium from polymer thermodynamics, the clustering penalty from the in vitro repair assays [6], and the MSH3 and FAN1 terms from the genetics and the mouse dose-response [4], [5].

Combining them as a product is a modelling choice rather than a published result, and it is the one piece of this structure with no direct support. It treats the steps as independent stages of a single pathway, so that the rate of a productive event is the product of the chances of each stage. That is reasonable if the stages really are sequential and non-interacting. It would be wrong if, for example, MutSβ bound at one site altered the slip-out probability at a neighbouring one, which tandem binding [3] makes conceivable. No available dataset separates these possibilities, so the product form is carried as an explicit structural assumption and named as such in the limitations.

TABLE I
PUBLISHED KINETIC ANCHORS USED FOR CALIBRATION.

Anchor	Published value
Mean transit time, 40 to 60 repeats	50 years
Mean transit time, 60 to 80 repeats	12 years
Mean transit time, 80 to 150 repeats	about 4 years
Rise in mutation rate across 70 to 90 repeats	at least 6-fold

Site multiplicity, n(L) = L / w. A single MutSβ molecule occludes about w = 17 repeats at a hairpin stem [3], so a longer tract can carry proportionally more. This is the weakest of the length dependences, growing linearly, and it turns out to contribute very little; it is retained because it is measured rather than assumed.

Slip-out occupancy, $\varphi(L)$ = 1 / (1 + exp($-\beta$(L $-$ L_{50}))). MutSβ binds a one-unit slip-out, so the substrate must be present before anything can happen. Treating slipped and unslipped registers as a two-state equilibrium whose free-energy difference falls linearly with tract length gives a logistic function of L. The parameter β is α/RT, where α = d(ΔG_slip)/dL per repeat, and L_{50} is the length at which the slipped state is half-populated. L_{50} acts only as a ceiling at very long tracts and is fixed at 400 repeats, a value varied in the sensitivity analysis.

Clustered-repair failure, (1 $-$ C) + R·C with C(L) = 1 / (1 + exp($-$(L $-$ L_{sw})/w_{sw})). Isolated slip-outs are repaired efficiently and clustered slip-outs are not [6]. C(L) is the fraction of slip-outs that occur in clusters, modelled as a logistic switch centred on L_{sw} with width w_{sw}, and R is the fold rise in the probability that a slip-out escapes to an expansion event once it is part of a cluster. Setting R = 1 recovers a model with no clustering effect.

Target abundance, θ and f. θ is relative MSH3 protein, entering the expression to the first power for reasons established in Section III-B. The divisor f is relative FAN1 activity, which protects.

Repeats gained per event are drawn from a geometric distribution with mean 4, following the four-repeat insertion recovered in the reconstituted system [5]. Contractions occur at a fixed fraction of the expansion rate with a mean of 2 repeats; both numbers are assumed and both are varied in Section III-M.

C. Calibration

Four parameters, $\lambda(40)$, β, R and L_{sw}, were fitted to four published anchors [1]:

The fourth is published as a lower bound and was used as an equality target, so the fitted model sits at the slowest acceleration consistent with the report. Transit times were computed from the mean-field integral T = $\int$ dL / (s(L)·$\lambda(L)$) and checked afterwards against the stochastic simulation. Residuals were taken in log space so that all four anchors carry equal weight, and minimised by multi-start trust-region least squares from 54 starting points. The fit is exact, with a residual cost of about 6×10^{-33}.

Five observations were kept out of the fit entirely and used only for validation: the fraction of projection neurons exceeding their inherited length, the fraction of neuronal life spent below 150 repeats, the duration of the terminal decline, the distribution of terminal tract lengths, and the relationship between CAP score and neuron loss. A sixth, the difference in expansion rate between direct and indirect pathway neurons, is used in Section III-K.

D. Feasible parameter ranges

With four parameters and four anchors the fit is exact, so there is no residual variance from which to construct a conventional confidence interval, and the anchors themselves are published without uncertainty. The question that can be answered instead is a profile question: holding one parameter fixed at a given value and refitting the other three, how badly does the worst anchor miss?

For each parameter, a grid of fixed values was scanned, the remaining three were refitted by bounded least squares from three starting points, and the maximum relative deviation across the four anchors was recorded. Feasible ranges are reported at three tolerances, 10%, 25% and 50%. These are not confidence intervals and are not described as such; they state how much freedom each parameter has while the model still reproduces the published kinetics.

E. Layer 2: stochastic simulation of neuron populations

Single neurons were simulated by an exact Gillespie algorithm over three events: expansion, contraction and elimination. The elimination hazard is zero below the 150-repeat threshold and rises logistically in L above it. It is written this way so that the short terminal decline is an emergent consequence of how fast the tract grows at long lengths rather than something imposed by hand.

Parameter sweeps need something faster. A vectorised tau-leaping scheme advances the whole population in steps of 1/24 year, drawing the number of events per step from a Poisson distribution and the total repeats gained from the corresponding negative binomial, which makes the step-size distribution exact and leaves only the constancy of λ within a step as an approximation. Treatment enters as a step change in θ at a given age; the exact solver advances precisely to that time and resamples, so the piecewise-constant rates stay exact there too.

Agreement between the two solvers was checked at three step sizes and two inherited lengths. The largest absolute difference between survival curves was 0.013 to 0.034, and ages at 50% loss agreed to within 1.1 years (`results/solver_agreement.csv`).

The tau-leaping solver also carries an optional non-cell-autonomous term, multiplying every surviving neuron's hazard by $(1 + k_{\mathrm{nca}} \cdot f_{\mathrm{lost}})$ where f_{lost} is the fraction of the population already eliminated. Setting k_{nca} to zero recovers the purely cell-autonomous model.

F. Layer 3: from MSH3 protein to expansion rate

Because MutSβ engagement enters λ as a product, a treatment that leaves a fraction m of MSH3 protein multiplies λ by m^k, where k is the number of MutSβ molecules in the productive species. That makes k measurable from a dose-response curve: a first-order species predicts proportional suppression, a tandem trimer predicts cubic suppression.

The published mouse regression [4] was used as the comparator, and k = 1, 1.5, 2 and 3 were scored by root-mean-square error over the full range of MSH3 levels. The two individual measured doses were then compared against the winning model as a check.

G. Direct and indirect pathway neurons

Indirect-pathway projection neurons are reported to expand about 15% faster than direct-pathway neurons, and to be depleted relative to controls, where they make up 47% (SD 6) of projection neurons [1]. Neither figure was used in calibration.

Two populations were simulated with rate multipliers of 1.00 and 1.15, combined in the control proportion, and the indirect-pathway share of surviving neurons was tracked against age and against overall neuron loss. The rate ratio was then scanned from 1.00 to 2.00 to establish how large a difference would be needed to produce a given depletion.

H. Genetic modifiers as parameter perturbations

Each known class of modifier maps onto a parameter of the rate law, which turns a genotype into a predicted shift in onset:

Each was scanned around its baseline and the shift in age at 25% neuron loss recorded for a 42-repeat carrier. These are predictions: no modifier effect size entered the calibration.

I. Delivery module

Exposure to response. An Emax curve, MSH3(%) = 100/(1 + (x/EC_{50})ˆh), was fitted to the two published mouse intracerebroventricular doses expressed per gram of brain, giving EC_{50} = 10.1 nmol g^{-1} and h = 1.22. Inverting it converts a target knockdown into a required tissue exposure.

Route A, CSF administration. Rather than invent a CSF transport model with parameters that cannot be checked, measured primate target engagement was used directly [7] and converted through the Emax curve into an implied striatal exposure. A tissue penetration length was then inferred from the reported gradient rather than assumed: with an exponential depth profile, a ten-fold difference over a depth difference of about 9 mm implies L_{pen} = Δdepth / ln(ratio) = 3.9 mm. Applying the same penetration length to a human putamen, whose depth is scaled by the cube root of brain mass from macaque to human, predicts how much worse the human case should be.

Route B, systemic nanocarrier. Transcytosis efficiency was modelled as the product of two opposing terms, surface capture and abluminal release:

$$\eta(K_D, v) = f_v \cdot \frac{K_{D,b}}{K_D + K_{D,b}} \cdot \frac{1}{1 + K_r/K_D} \qquad (2)$$

TABLE II
MAPPING OF KNOWN GENETIC MODIFIERS ONTO PARAMETERS OF THE FITTED RATE LAW.

Modifier	Parameter	Direction
MSH3 expression	θ	more MSH3 is faster
FAN1 activity	f	more FAN1 is slower
Loss of a CAA interruption	L_{sw}	an uninterrupted tract reaches the clustered regime at shorter total length, so L_{sw} falls

The first term rises as affinity rises, the second falls, and the product peaks at $K_D = \sqrt{(K_{D,b} \cdot K_r)}$. The two constants were fitted to the two bivalent variants of the measured anti-transferrin-receptor gold nanoparticle series, and the valency factor f_v was taken from the monovalent to bivalent ratio at matched affinity [8]. The required systemic dose follows from the target tissue exposure, the parenchymal accumulation and an assumed endosomal escape fraction, which is swept because it is the least constrained number in the calculation.

J. Dosing schedules

Section II-F establishes that λ is first-order in MSH3. That has a convenient consequence: over decades, the cumulative effect of an oscillating MSH3 level depends only on its time average, not on the shape of the waveform. The correction from an idealised step change to a real dosing schedule is therefore exact rather than approximate.

Tissue concentration was modelled as first-order elimination with repeated bolus dosing, with steady-state accumulation handled analytically. MSH3 follows from the Emax curve, and the time average over one steady-state cycle gives the effective residual MSH3 that is fed back into the lifetime model. Neither the CNS half-life of a stabilised oligonucleotide nor the eventual dosing interval is settled, so both were swept rather than assumed: half-lives of 1 to 12 months, intervals of 3 to 12 months.

K. In silico trial

Virtual cohorts of 9,000 projection neurons carrying a given inherited length were simulated with a step change in striatal MSH3 at a given age, held for life. Scenarios were anchored to measured achievable engagement, with residual MSH3 of 0.71, 0.57 and 0.45, and to aspirational levels of 0.25, 0.10 and 0.05.

Two endpoint families were used. Neuron loss at ages 70 and 80 is uncensored and always available. Age at 25% and 50% loss is right-censored at 110 years: under strong treatment the cohort never reaches the threshold, and naive interpolation silently returns the last simulated time point, which inflates apparent benefit. Censored values are reported as lower bounds and marked as such in every table and figure. Monte Carlo noise, estimated from zero-knockdown control arms that should show no effect, is about ± 1 year on the age endpoints.

L. Sample size for a real trial

The in silico trial was converted into a conventional power calculation for a randomised, placebo-controlled study whose endpoint is the change in projection neuron survival over the trial period, the quantity for which striatal volume on MRI is a proxy.

Each participant contributes $\mathrm{loss}(\mathrm{age}_0 + \mathrm{T}) - \mathrm{loss}(\mathrm{age}_0)$ in percentage points. Between-subject variance has two sources, kept separate: the spread of inherited CAG lengths across the enrolled cohort, modelled as a triangular distribution over 40 to 50 repeats centred on 44, and measurement error on the imaging endpoint, which is not a property of this model and is swept from 0 to 4 points. Sample size follows from $n = 2(z_{1-\alpha/2} + z_{1-\beta})^2 \ \sigma^2/\delta^2$ at $\alpha = 0.05$ and 80% power, with two measurements per participant.

M. Global sensitivity analysis

Fifteen inputs were varied simultaneously over a 160-sample Latin hypercube: six published anchors, including the step size and the pharmacodynamic order, and nine assumed structural constants. The rate law was recalibrated inside every sample, so calibration uncertainty is propagated rather than held fixed at the central fit. Sensitivity was quantified by partial rank correlation coefficients, which suit monotonic but non-linear responses, and Spearman correlations were computed alongside as a check.

N. Where every parameter comes from

III. RESULTS

A. The clock is a switch, not a ramp

The four-parameter rate law reproduces all four published kinetic anchors exactly (Fig. 1C; `results/calibration_check.csv`). The fitted basal rate is $\lambda(40) = 0.070$ events per cell per year, which is about one mutation every fourteen years, rising to 8.1 per year at 150 repeats. That is a 117-fold increase over the length range a neuron traverses in a lifetime (Fig. 1B).

How the increase is assembled turns out to matter more than its size. Almost all of it is carried by one term. Half of slip-outs are clustered by the time the tract reaches $L_{\mathrm{sw}} = 80.2$ repeats, and a clustered slip-out is R = 17.7 times more likely to escape repair and become a real expansion event than an isolated one (Fig. 1A). The other two length-dependent terms barely move. Site multiplicity n(L) rises linearly and only triples across the whole range, and the thermodynamic occupancy term is close to flat: the fitted α works out to 0.0036 kcal per mole per repeat, which is a few hundredths of the thermal energy scale and well inside the noise of any stability measurement.

The profile analysis makes the point sharply (Fig. 2; `results/profile_intervals.csv`). Holding the fit to

TABLE III

PROVENANCE OF EVERY PARAMETER IN THE MODEL. ENTRIES MARKED *fitted here* WERE ESTIMATED IN THIS WORK; ENTRIES MARKED *assumed* CARRY NO SUPPORTING MEASUREMENT AND ARE ALL VARIED IN THE SENSITIVITY ANALYSIS.

Symbol	Meaning	Value	Provenance
w	MutSβ stem footprint	17 repeats	measured [3], preprint
step	repeats gained per event	geometric, mean 4	measured in vitro [5]
L_C	commitment threshold	150 repeats	measured [1]
T(40,60), T(60,80), T(80,150)	mean transit times	50, 12, 4 y	model-based estimates [1]
$\lambda(90)/\lambda(70)$	rate acceleration	at least 6	estimate [1]
iSPN / dSPN rate ratio	subtype difference	1.15	estimate [1], held out
iSPN share in controls	subtype baseline	47% (SD 6)	measured [1], held out
PD regression	expansion prevented against MSH3	intercepts 92.4, 110.4	measured in mouse [4]
EC_{50}, h	exposure to response	10.1 nmol g^{-1}, 1.22	**fitted here** to [4]
$\lambda(40)$	basal mutation rate	0.070 y^{-1}	**fitted here**
β (α)	slip-out stabilisation	0.0062 repeat^{-1} (0.0036 kcal mol^{-1})	**fitted here**, not identifiable
R	clustered to isolated escape ratio	17.7	**fitted here**
L_{sw}	clustering switch midpoint	80.2 repeats	**fitted here**
$K_{D,b}$, K_r	capture and release constants	286, 64 nM	**fitted here** to [8]
f_v	bivalent valency penalty	0.174	measured [8]
w_{sw}, L_{50}, L_{death}, width, $h_{\max}$, contraction terms	structural constants	see `fitted_parameters.json`	**assumed**, all swept
brain and striatum masses, molecular weight, escape fraction, depths	scaling constants	see `delivery_constants.csv`, `translation.csv`	**assumed**, all swept
tissue half-life, dosing interval	schedule	1 to 12 mo, 3 to 12 mo	**swept**, not assumed
imaging measurement SD	trial noise	0 to 4 points	**swept**, not assumed

within 10% of its best worst-case residual, $\lambda(40)$ is pinned to 0.060 to 0.071 per year and L_{sw} to 78.3 to 81.7 repeats, but β ranges over the entire grid from 0.0001 to 0.03 per repeat without the fit degrading at all. Loosen the tolerance to 25% and L_{sw} still only widens to 71.7 to 118.3 repeats while R spans 2 to 42. In other words: **the position of the switch is one of the best-determined quantities in the model, and the slip-out stability parameter is not determined at all.** β is consistent with zero, and setting it to zero costs nothing. The model reduces, in practice, to a threshold at roughly eighty repeats.

That is a claim about mechanism, not a curve-fitting observation. It says the acceleration seen clinically between seventy and ninety repeats is not a slow thermodynamic drift in how stable a slip-out is. It is the point at which the tract first becomes long enough to hold two or more slip-outs close together, which is exactly the configuration that human MutSβ was shown to repair badly in vitro [6]. The model did not know about that experiment. It was asked only to fit transit times, and it put its switch where the biochemistry says the substrate changes.

One corner of the earlier identifiability scan is degenerate and should be read as a warning rather than a result. Forcing the 80 to 150 repeat transit down to two years drives L_{sw} to the top of its range and R to roughly 2,600. That variant is the parameterisation breaking down, not a biological estimate, and it is flagged as such in `results/identifiability_profiles.csv`.

B. The productive MutSβ species is first-order in MSH3

If an expansion event needed a tandem assembly of k MutSβ molecules sitting on the same tract, then halving MSH3 would cut the rate by a factor of 2^k, not 2. That is a sharp, testable difference, and there is published data that bears on it.

The mouse dose-response is close to directly proportional, with a regression slope near unity [4]. Scoring the three stoichiometries against that regression gives a root mean square error of 4.97% for k = 1, 18.02% for k = 2 and 26.43% for k = 3 (Fig. 3; `results/model_comparison_msh3.csv`). The two measured points land almost on top of the first-order prediction. At 25% residual MSH3 the measured prevention is 78.1%, first order predicts 75.0%, and third order predicts 98.4%, which is not close.

This needs to be stated carefully, because it sits in apparent tension with the cryo-EM proposal that the roughly forty-repeat pathogenic threshold is where three or more MutSβ molecules can first bind at once [3]. The two accounts are reconcilable, and the reconciliation is specific: tandem binding may set which substrates are competent, while the rate-limiting productive step still involves a single MutSβ and MutLγ complex acting at one of those sites. Occupancy and catalysis are different questions, and the data here speak to the second.

The practical consequence cuts both ways. There is no cooperative threshold to clear, so even modest MSH3 lowering buys proportional suppression from the first percent. But there is also no cooperative bonus, so near-complete suppression needs near-complete lowering. Everything in the delivery and trial analysis below follows from that one fact.

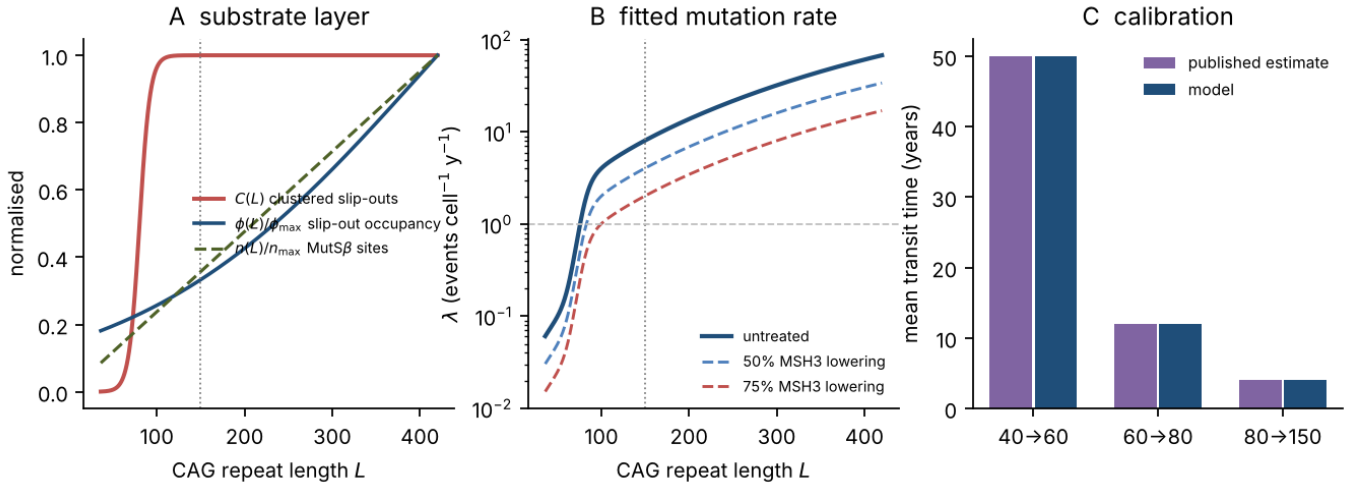


Fig. 1. **The fitted expansion rate law.** (A) The three length-dependent terms plotted separately. The clustered fraction C(L) switches sharply near eighty repeats; occupancy $\varphi(L)$ and site multiplicity n(L) grow slowly and almost linearly. The dotted line marks the 150-repeat commitment threshold. (B) Fitted $\lambda(L)$ on a log scale, with the effect of 50% and 75% MSH3 lowering. (C) Model against published mean transit times.

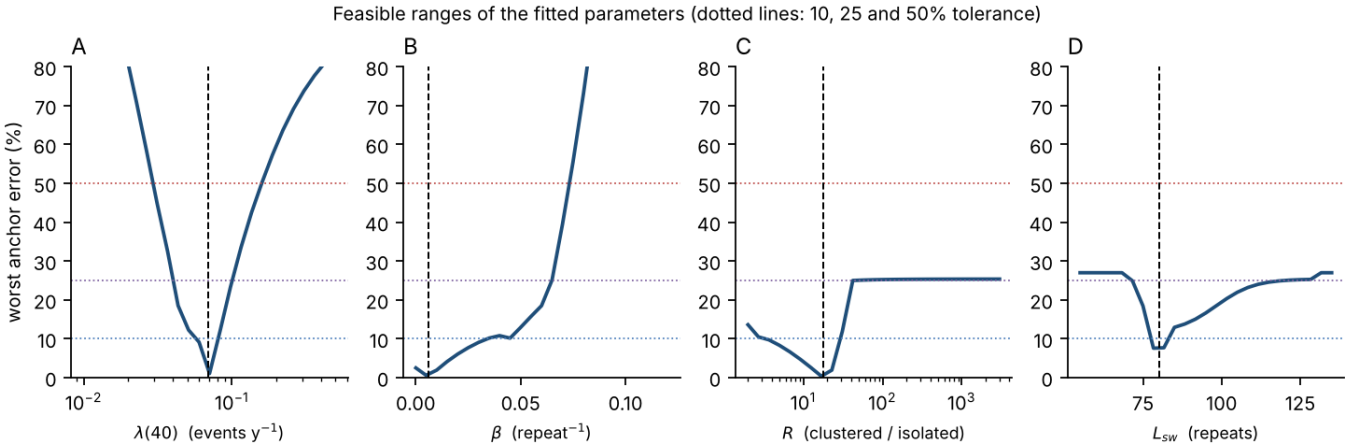


Fig. 2. **What the data actually pin down.** Each panel holds one parameter at a fixed value, refits the other three, and plots the worst relative deviation from the four kinetic anchors. Flat regions are directions the data cannot see. $\lambda(40)$ and L_{sw} sit in sharp wells; β is flat across four orders of magnitude; R has a broad floor with a soft lower edge.

C. The population model reproduces four of five held-out observations

Simulating 20,000 independent projection neurons carrying 42 repeats, with no parameter tuned to any of the quantities below (`results/population_validation.csv`):

Four of the five come out right. The terminal length distribution is too narrow: the model's upper tail stops at 375 where lengths beyond 500 are observed. That is expected and has a known cause, since the simulation caps tract length at 1,000 repeats and uses a step size that does not grow with length. A length-dependent step would stretch the tail without changing anything upstream, and it was tried and discarded during calibration because it made the kinetic fit worse rather than better.

The fifth row is a genuine failure, and Section III-D and 3.5 are about chasing it down rather than explaining it away.

D. Accelerating neuron death does not close the gap

Donors with CAP scores above 600 are reported to have lost 80 to 99% of their projection neurons [1]. The model loses 43%. That is not a small discrepancy and it deserves a real attempt at explanation.

The obvious candidate is that striatal degeneration is not purely cell-autonomous, and that losing neurons makes the survivors more vulnerable. The network is losing inhibitory tone, trophic support and glutamate buffering as it thins, so a late avalanche is biologically plausible. This was tested directly, by multiplying every survivor's elimination hazard by $(1 + k_{\mathrm{nca}} \cdot f_{\mathrm{lost}})$ and scanning the coupling constant from zero to forty.

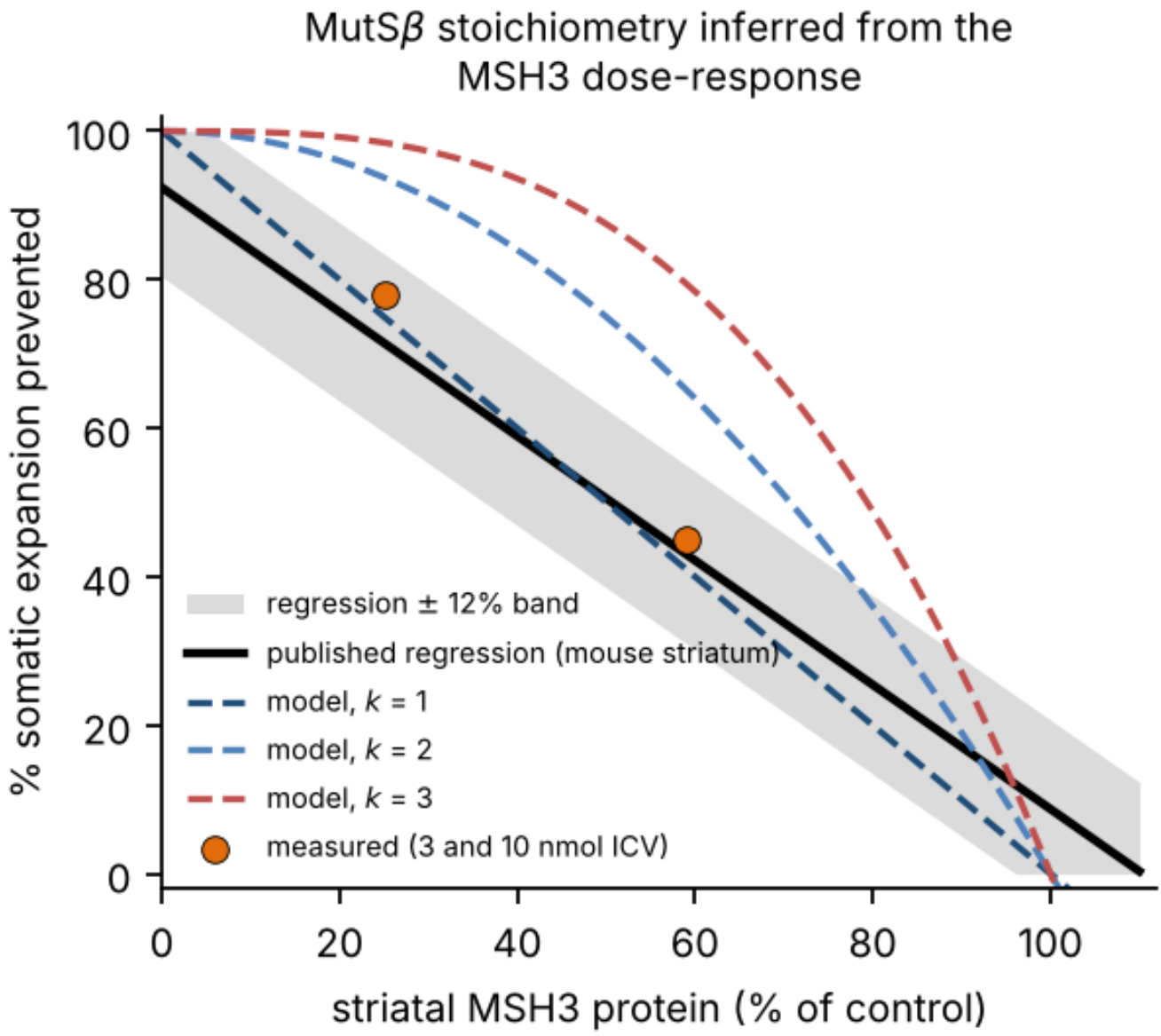


Fig. 3. **Inferring MutSβ stoichiometry from the MSH3 dose-response.** The published regression (black) with the two measured intracerebroventricular doses (orange), against model predictions for k = 1, 2 and 3 simultaneously required MutSβ molecules.

TABLE IV
HELD-OUT OBSERVATIONS, NONE OF WHICH WAS USED IN CALIBRATION, AGAINST THE CALIBRATED POPULATION MODEL.

Held-out observation	**Published**	**Model**
Neurons expanding beyond the inherited length	95 to 98%	99.9%
Fraction of neuronal life spent between 42 and 150 repeats	96.4% (SD 2.0)	97.5%
Time from 150 repeats to elimination	months, not decades	20 months (median)
Terminal tract lengths	100 to over 500, maximum 842	median 236, 97.5th percentile 375
Neuron loss at CAP = 600	80 to 99%	**43.5%**

It does essentially nothing. Mean loss at CAP 600 moves from 43.3% to 45.8% across the entire range (Fig. 5B; `results/knca_fit.csv`), and the reported range is never reached at any value. Pushing the coupling higher does not help either, because the mechanism is saturating against a hard structural constraint.

The reason is the clearest mechanistic statement this model makes. Once a neuron crosses 150 repeats it is dead within

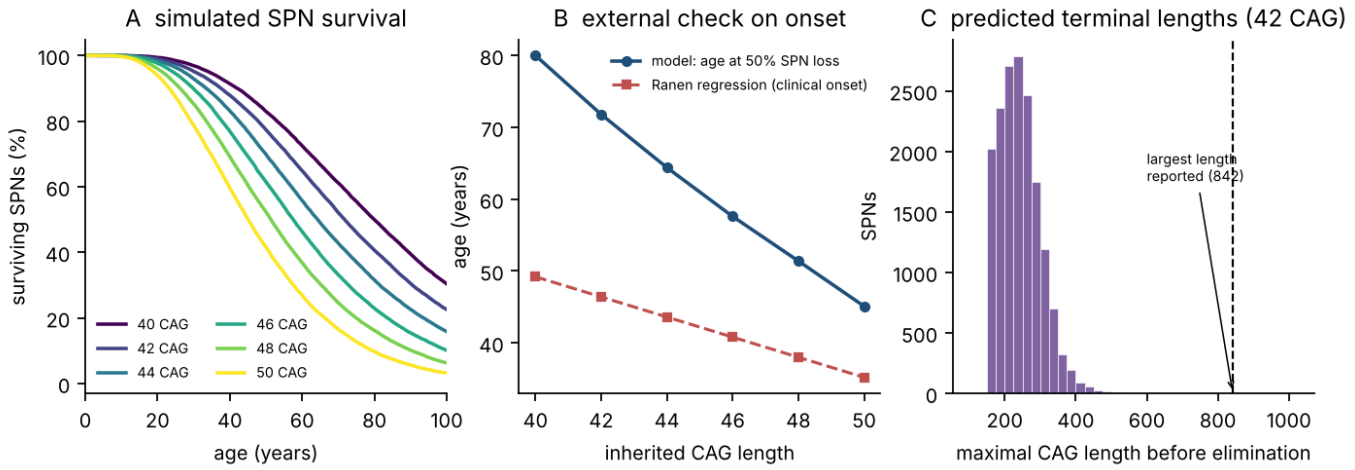


Fig. 4. **Simulated neuron populations.** (A) Survival curves by inherited CAG length. (B) Model age at 50% loss against an external clinical regression. (C) Distribution of terminal tract lengths for a 42-repeat allele.

about twenty months. The time spent waiting to reach that threshold is 97.5% of its life. Anything that acts after threshold is operating on the last 2.5% of the timeline, so it has almost no leverage on when the striatum is lost. Multiply the terminal hazard by forty and you shorten twenty months to a few weeks, which is invisible against five decades of waiting.

The same arithmetic applies to therapy, and that is why the delivery analysis matters as much as it does. There is no late rescue available in this model, for pathology or for treatment. Everything happens before the threshold or not at all.

E. What would close the gap, and why the slower model is kept anyway

Two structural possibilities remain. Either the tract commits a neuron to death at a shorter length than 150 repeats, or expansion is uniformly faster than the published dwell times imply. A two-dimensional scan over both settles which of them could work (Fig. 6A; `results/reconciliation_map.csv`).

Lowering the commitment length from 150 to 90 repeats moves end-stage loss only from 43.9% to 48.6%, so that is not the answer. A uniform rate multiplier g moves it from 43.9% at g = 1 to 90.8% at g = 2, so that is. Fitting the multiplier gives g = 1.96, which would imply transit times of 25.5, 6.1 and 2.0 years in place of the published 50, 12 and 4.

The trouble is that the faster model then fails an entirely different test, one that was not used to build either version. Under g = 1.96 a 42-repeat carrier reaches 50% neuron loss at age 36.9, and has lost 70.7% of projection neurons by the age an external clinical regression predicts for motor onset, 46.4 years [10]. For a 50-repeat carrier the figure is 81.9%. Losing four-fifths of the striatum before the first clinical sign is not credible, and it also contradicts the observation in the source paper that even at 25% loss more than nine out of ten surviving neurons still carry a tract that is not yet harmful [1].

The dwell-time-faithful model puts loss at predicted clinical onset at 18.4% for 42 repeats, 24.4% for 46 and 31.2% for 50. Those numbers are consistent with the single-cell picture and with the general finding that striatal atrophy on imaging precedes diagnosis by years rather than decades.

So the resolution adopted here is that “CAP above 600” in the source describes a late-stage, severely affected autopsy population rather than the threshold value CAP = 600 at which the model was evaluated. Post-mortem series are not sampled uniformly along the CAP axis; they are enriched for people who died of advanced disease. Evaluating the model at the stated threshold is the strictest possible reading of the published sentence, and a more generous reading dissolves much of the discrepancy.

This is an unresolved tension in the published literature rather than a modelling artefact, and it is worth being plain about that. The dwell-time-faithful model is used for every therapeutic prediction in this paper and the faster one is carried as a sensitivity bound. The tension is also testable: the two versions predict visibly different loss-versus-CAP trajectories (Fig. 12B), and a post-mortem series with enough cases spread across the CAP range would separate them without needing any new technique.

F. A held-out prediction: which neurons go first, and by how much

Indirect-pathway projection neurons are reported to expand about 15% faster than direct-pathway neurons, and to make up a smaller share of survivors in Huntington’s disease donors than in controls, where they are 47% with a standard deviation of 6 [1]. Neither number was used in calibration, so running the two subtypes as separate populations with a 1.15 rate ratio produces a prediction with nothing fitted to it.

The prediction is that the depletion is real, orderly, and modest (Fig. 7; `results/subtype_trajectory.csv`). For a 42-repeat carrier the indirect-pathway share of survivors

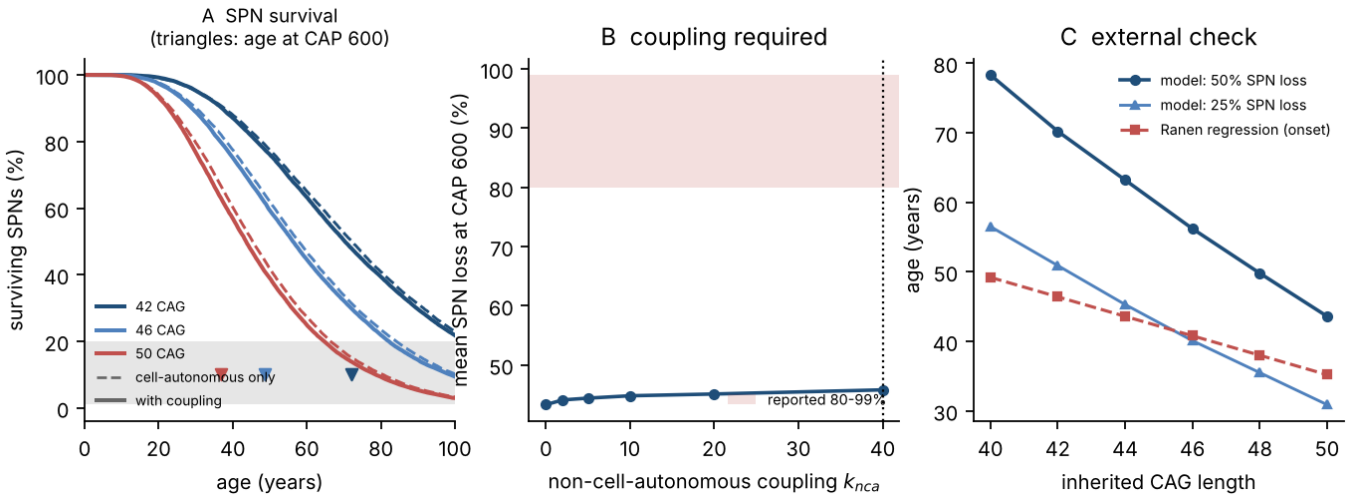


Fig. 5. **Testing non-cell-autonomous acceleration.** (A) Survival with (solid) and without (dashed) coupling; triangles mark the age at CAP 600. (B) Mean neuron loss at CAP 600 against the coupling constant, with the reported range shaded. It is never reached. (C) Model ages at 25% and 50% loss against the external onset regression.

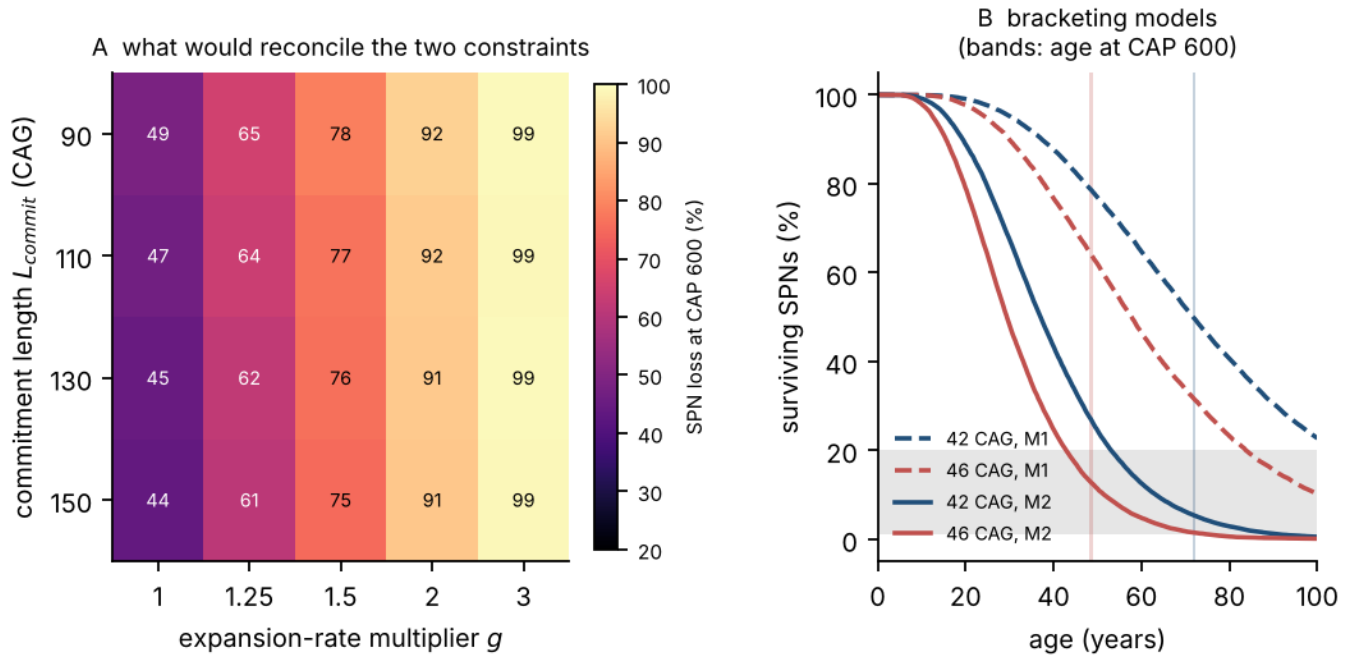


Fig. 6. **Reconciling two published constraints.** (A) Mean neuron loss at CAP 600 across expansion-rate multipliers and commitment lengths. Only a roughly two-fold faster clock reaches the reported range. (B) Survival under the two bracketing parameterisations.

falls from 47% to 45.9% by the time 10% of neurons are gone, to 44.2% at 25% loss, to 41.1% at 50% loss and to 36.3% at 75% loss. The same figures come out for 46 and 50 repeats within a few tenths of a point, which is a useful result in itself: **subtype depletion tracks total neuron loss, not age and not inherited CAG length.** It is a readout of disease stage rather than of genotype.

Two things follow. First, a depletion of five or six percentage points at half loss is small against a control standard deviation of six, so a cross-sectional study would need a lot of donors and careful staging to see it. The model says the effect is there but says it is not easy to measure, and that is a more useful statement than a bare direction.

Second, the inverse scan puts bounds on the rate difference (Fig. 7C; `results/subtype_rate_scan.csv`). A ratio of 1.00 gives a depletion of −0.2 points, which is the Monte Carlo noise floor. The reported 1.15 gives 6.0 points at half loss. Reaching twenty points, the sort of difference that would

be unmistakable in autopsy tissue, would need a ratio of about 1.6, which is well outside what the sequencing data support. So if a future series reports a large indirect-pathway depletion, a 15% rate difference will not account for it, and something besides expansion rate, such as a genuine difference in vulnerability per repeat, would have to be invoked.

G. Genetic modifiers land on three different parameters, and only one of them is druggable

The strongest known modifiers of age at onset in Huntington's disease sit in mismatch-repair genes and in the sequence of the CAG tract itself. Each maps onto a distinct parameter of the fitted rate law, which means the model can convert a genotype into a predicted shift in years without any new fitting (Fig. 8; `results/modifier_effects.csv`). Baseline for a 42-repeat carrier is age 51.7 at 25% neuron loss.

MSH3 expression enters as the first-order factor θ. Dropping it to 60% of normal, roughly what a protective expression quantitative trait locus might do, delays 25% loss by 33.6 years. Raising it to 150% brings the same endpoint forward by 16.6 years. The curve is steep and strongly asymmetric: the first 10% of lowering buys 5.6 years, the next 10% buys another 6.3, and the slope keeps rising. That asymmetry is the compounding effect, and it is the reason partial knockdown is worth more than it looks.

FAN1 activity enters as the protective divisor f. Halving it advances onset by 25.9 years, and doubling it delays onset by 51.5 years. FAN1 is numerically the strongest single lever in the model, which fits its position as one of the strongest onset modifiers in human genetics, and it fits the recent demonstration that FAN1 opposes MutLγ directly at the incision step [5]. It is also, unfortunately, the hardest to use: increasing an enzyme's activity in a deep brain structure is a much harder pharmacological problem than reducing a protein's abundance.

Loss of a CAA interruption enters as a downward shift in the clustering switch L_{sw}, because an uninterrupted tract reaches the clustered regime at a shorter total length than an interrupted one of the same nominal size. Shifting the switch down by ten repeats advances 25% loss by 16.8 years, and by fifteen repeats advances it by 24.2 years. This gives a quantitative mechanism for an observation that is otherwise hard to rationalise, namely that two carriers with identical nominal CAG counts can differ by a decade or more in onset depending on whether the tract is pure.

All three of these are predictions rather than fits. No modifier effect size entered the calibration, and the model was never shown a single human onset age.

H. Delivery: the striatum is the wrong place to reach from cerebrospinal fluid

Converting the measured primate target engagement through the fitted exposure-response relationship (Fig. 9A) gives what an intrathecal oligonucleotide actually achieves in the structure that matters:

Two cautions belong with that table. These figures come from 25 mg given intrathecally three times in macaques, a regimen the original authors themselves note likely exceeds what clinical dosing would deliver [7]. And the inferred tissue penetration length is 3.9 mm, which is short against the depth of the human putamen. Scaling putaminal depth from a roughly 90 g macaque brain to a roughly 1,300 g human brain by the cube root of mass moves the relevant depth from about 10 mm to about 14.8 mm, which predicts a further 3.4-fold attenuation in humans. Cube-root geometric scaling is crude and the magnitude of that correction should not be taken literally, but its direction is not in doubt. A diffusion-limited route into a deep structure gets worse, not better, when the brain gets bigger.

So the honest summary is that the best measured intrathecal engagement in the right cell type in the right structure is 29%, that figure comes from a heavier-than-clinical regimen in a smaller brain, and the model says it corresponds to roughly a third of the available expansion suppression.

I. A systemic nanocarrier has an affinity optimum, and the optimum is not enough

Fitting the two-step capture and release model to the published anti-transferrin-receptor gold nanoparticle affinity series gives $K_{D,b}$ = 286 nM for luminal capture and K_r = 64 nM for abluminal release, so parenchymal accumulation peaks at K_D of about 135 nM (Fig. 10A). That is an order of magnitude weaker than the high-affinity variants that are usually engineered, and it reproduces the familiar receptor-mediated transcytosis paradox from data rather than by assertion: binding tighter captures more carrier at the luminal face but releases less of it on the far side, and the product of the two has an interior maximum. Valency matters more than affinity over the measured range, with a fitted penalty of 5.7-fold for bivalent relative to monovalent binding.

The optimum is real and it is small. Moving from the best measured monovalent design, at 0.230% of injected dose per gram, to the modelled optimum gives 2.0-fold more drug in the parenchyma. Set against the dose requirement, that is not close to sufficient (`results/translation.csv`):

The contrast that makes the point is this. Seven milligrams of oligonucleotide inside a 20 g human striatum would achieve 75% MSH3 lowering. Reaching that by the systemic transcytosis route would require injecting 7.6 grams. Affinity and valency engineering closes a factor of two of that gap. The rest has to come from somewhere else, and the arithmetic says exactly where to look: endosomal escape, assumed here at 2%, scales the required dose linearly and is by far the least constrained number in the chain. Receptor choice is the next lever. Failing both, the systemic route is the wrong route, and intraparenchymal or convection-enhanced delivery becomes the comparison to beat.

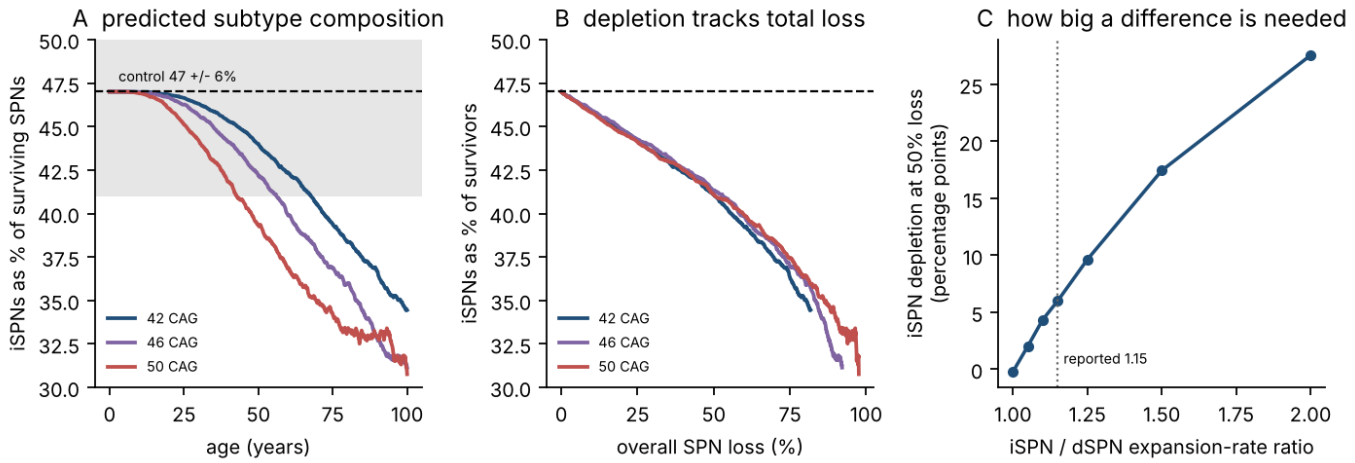


Fig. 7. **Predicted subtype composition.** (A) Indirect-pathway neurons as a percentage of survivors over age, for three inherited lengths, against the control value of 47% with its measured spread shaded. (B) The same curves plotted against total neuron loss: they collapse onto one line, so depletion reports stage rather than genotype. (C) Depletion at half loss against the assumed rate ratio, with the reported value marked.

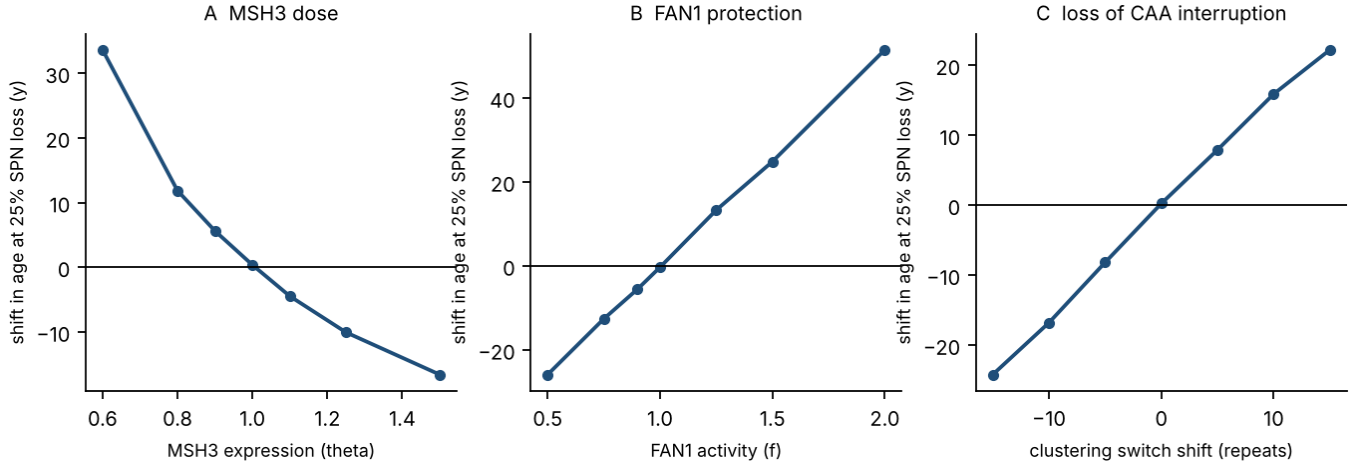


Fig. 8. **Genetic modifiers as perturbations of one rate law.** Shift in age at 25% neuron loss for a 42-repeat carrier when (A) MSH3 expression, (B) FAN1 activity, or (C) the clustering switch position is perturbed. Zero on each vertical axis is the unperturbed baseline.

TABLE V
TARGET ENGAGEMENT ACHIEVED BY INTRATHECAL DELIVERY IN PRIMATE STRIATUM, AND THE EXPANSION SUPPRESSION IT IMPLIES.

Region	Knockdown achieved	Implied exposure	Predicted expansion suppression
Putamen, projection neurons	29%	4.8 nmol per g	**33%**
Putamen, bulk tissue	25%	4.1 nmol per g	30%
Caudate, single-nucleus RNA sequencing	43%	8.0 nmol per g	45%
Caudate, quantitative PCR	55%	11.9 nmol per g	55%

TABLE VI
SYSTEMIC DOSE REQUIRED FOR 75% STRIATAL MSH3 LOWERING, BY NANOCARRIER DESIGN.

Design	Dose for 75% striatal MSH3 lowering	Improvement needed against 3 mg per kg
Bivalent, K_D 21 nM	625 mg per kg	208-fold
Monovalent, K_D 22 nM (best measured)	109 mg per kg	36-fold
Monovalent, at the modelled optimum	54 mg per kg	18-fold

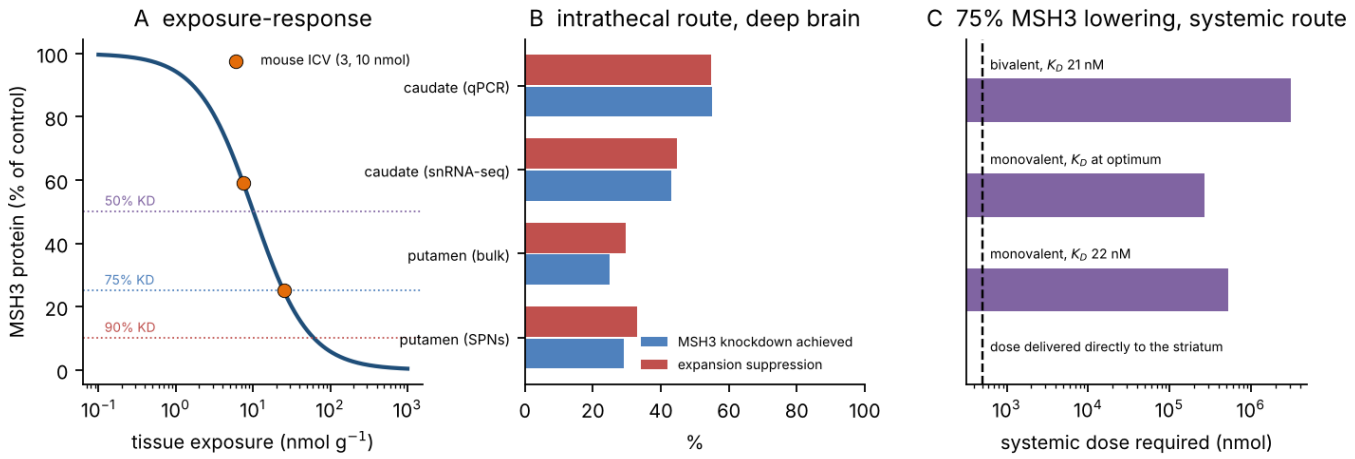


Fig. 9. **The delivery problem.** (A) Exposure-response fitted to the two mouse intracerebroventricular doses, with target knockdown levels marked. (B) What intrathecal delivery achieves in deep brain and the expansion suppression it implies. (C) Systemic dose required for 75% MSH3 lowering, against the dose that actually needs to arrive in the striatum (dashed).

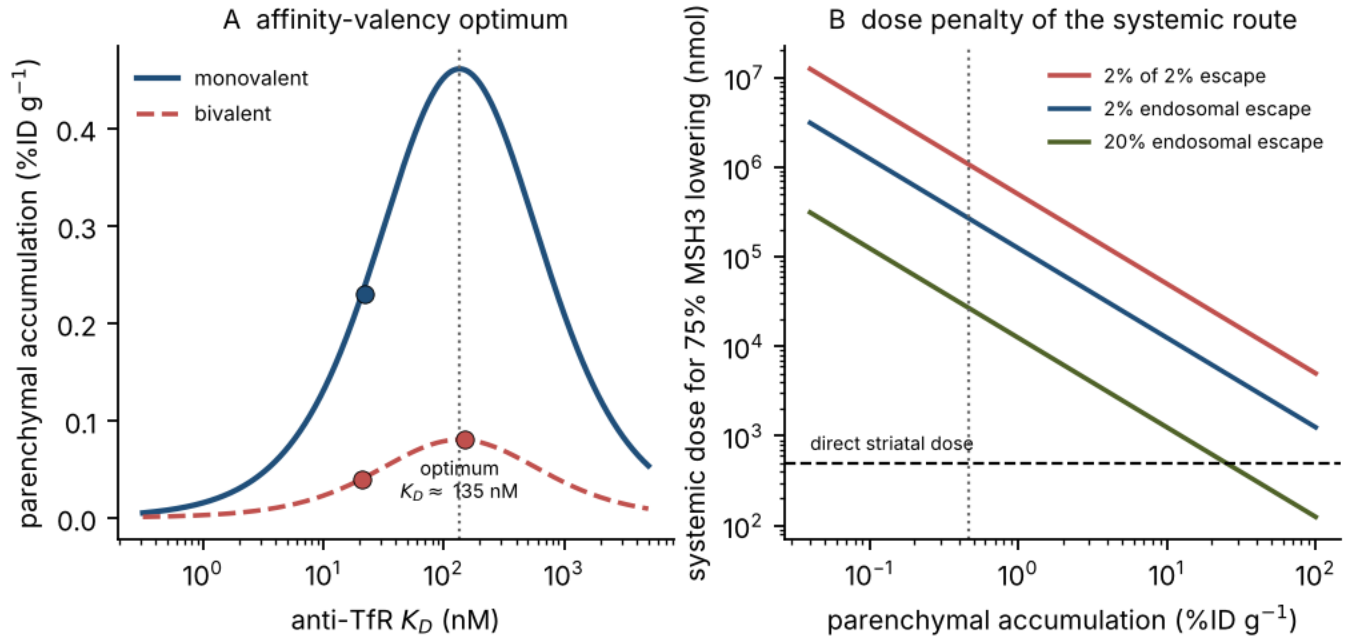


Fig. 10. **Nanocarrier design space.** (A) Fitted affinity and valency response against the measured gold nanoparticle data, with the optimum at K_D of about 135 nM. (B) Systemic dose required for 75% MSH3 lowering as a function of parenchymal accumulation, for three endosomal escape fractions.

J. In silico trial: a real benefit, sublinear in dose, and dominated by timing

For a 42-repeat carrier, 47.3% of projection neurons are lost by age 70 without treatment. With lifelong treatment starting at age 30 (Fig. 11A; `results/trial_grid.csv`):

The zero-knockdown arm is there deliberately. It should show no effect, and it shows −0.4 years, which calibrates the Monte Carlo noise on these endpoints at roughly one year. The two censored rows are not predictions of a 57-year delay. They mean the simulated cohort never reaches 25% loss within a 110-year lifespan, and the number is a lower bound forced by where the simulation stops.

Three features of this table matter clinically.

Timing dominates everything. The same 29% knockdown delays 25% loss by 12.6 years if started at age 20, 8.2 years at 30, 3.8 years at 40 and 0.0 years at 50 (Fig. 12A). Loss at 70 still falls from 47.3% to 40.0% in the age-50 arm, so the drug is doing something, but the clinically meaningful endpoint has already gone past. This is Section III-D in therapeutic clothing: the drug acts only on the pre-threshold clock, and by age 50 a substantial share of neurons have already committed and cannot be recalled.

TABLE VII
PREDICTED OUTCOME OF LIFELONG TREATMENT FROM AGE 30 IN A 42-REPEAT CARRIER. CENSORED ENTRIES ARE LOWER BOUNDS SET BY THE SIMULATED LIFESPAN.

Striatal MSH3 knockdown	Neuron loss at 70	Delay to 25% loss
0% (control arm)	47.2%	−0.4 y (Monte Carlo noise)
29% (measured, putaminal neurons)	**33.6%**	**8.2 y**
43% (caudate, sequencing)	26.2%	15.7 y
55% (caudate, PCR)	21.1%	25.0 y
75% (aspirational)	12.6%	over 57.5 y (censored)
95%	6.6%	over 57.5 y (censored)

A longer inherited allele compresses the window. The same intervention from age 30 delays 25% loss by 10.8 years at 40 repeats, 8.6 at 42, 4.8 at 45 and 2.9 at 48 (`results/trial_scenarios.csv`; the independently seeded grid run above gives 8.2 rather than 8.6 at 42 repeats, which is a fair indication of the Monte Carlo spread). This is an awkward result for trial design. The carriers most likely to be enrolled, because they convert soonest and give an answer fastest, are the carriers for whom partial MSH3 lowering helps least.

Returns rise steeply with knockdown. Going from 29% to 55% roughly triples the delay. Going to 75% pushes the cohort past the end of the simulated lifespan. The cause is compounding: the clock is slowed every year for decades, so the benefit multiplies rather than adds. The therapeutic value of closing the last part of the delivery gap is much larger than the size of the gap suggests, and that is an argument for continuing to engineer delivery rather than settling for what intrathecal dosing currently reaches.

K. From an idealised step to a real dosing schedule

The trial above assumes treatment sets striatal MSH3 to a new level instantly and holds it there for life. No real regimen does that. An intrathecal oligonucleotide is given as repeated boluses that decay between doses, so achieved knockdown oscillates between a peak and a trough.

Because the rate law is first-order in MSH3 (Section III-B), this correction is exact rather than approximate. The cumulative effect over decades depends only on the time-averaged MSH3 level, not on the shape of the waveform, so computing the time average over one steady-state dosing cycle and feeding it back as an effective residual gives the right lifetime answer. If the rate law had been cooperative, the peaks would have counted for more than the troughs and no such reduction would have been available. This is a case where a structural result from one part of the analysis buys exactness in another.

Neither the central-nervous-system half-life of a stabilised oligonucleotide nor the eventual dosing interval is settled, so both were swept rather than assumed (Fig. 13B; `results/dosing_timeaverage.csv`). With a per-dose exposure chosen to give 75% peak knockdown, the time-averaged knockdown ranges from 13.7% in the worst case tested (one-month half-life, annual dosing) to 96.2% in the best (twelve-month half-life, three-monthly dosing). The structure of that grid is simple and useful: what matters is the ratio of half-life to interval, and once the half-life exceeds the interval, accumulation carries the trough up and the average approaches the peak.

Running representative schedules back through the lifetime model, for a 42-repeat carrier treated from age 30 against 48.3% untreated loss at age 70 (`results/dosing_benefit.csv`):

The first row is the idealisation and the rest are what a schedule would actually deliver. Two of them beat the idealisation, which looks odd until you notice why: accumulation across doses lifts the steady-state peak above the single-dose peak, so a long half-life with frequent dosing ends up averaging better than a flat 75%. The row that should worry a trial designer is the third from bottom. Annual dosing with a three-month half-life loses 15 of the 35 available percentage points, not because the drug is weaker but because the patient spends most of the year near baseline. **The dosing interval is a therapeutic variable of the same order as the dose itself**, and it is one that can be optimised without any new chemistry.

L. Could a trial of plausible size and length see the effect?

A predicted benefit is only useful if it is detectable. Converting the lifetime model into a conventional sample-size calculation, with the endpoint being change in projection-neuron survival over the trial period (the quantity striatal volume on MRI stands in for), gives the following for a cohort enrolled at age 40 with an imaging measurement standard deviation of 2 percentage points (Fig. 14; `results/trial_power.csv`):

The headline is that this is a feasible trial. At the knockdown that has actually been measured, a three-year study with 133 participants per arm has 80% power, and that is well within the range of trials that have been run in this disease. The effect size is small in absolute terms, 1.02 percentage points of striatum over three years, but the between-subject variability from the spread of inherited CAG lengths is also small, 0.86 points, so the signal-to-noise ratio is workable.

What dominates the sample size is not biology but imaging precision. Holding everything else fixed and sweeping the measurement standard deviation from 0 to 4 points moves the required sample size from 12 to 498 per arm, a forty-fold range (Fig. 14C). The biological effect is the same in every one of those cases. **Measurement error is the binding constraint on trial size, not the strength of the drug effect**, and improving the reproducibility of the imaging endpoint is worth as much as improving target engagement.

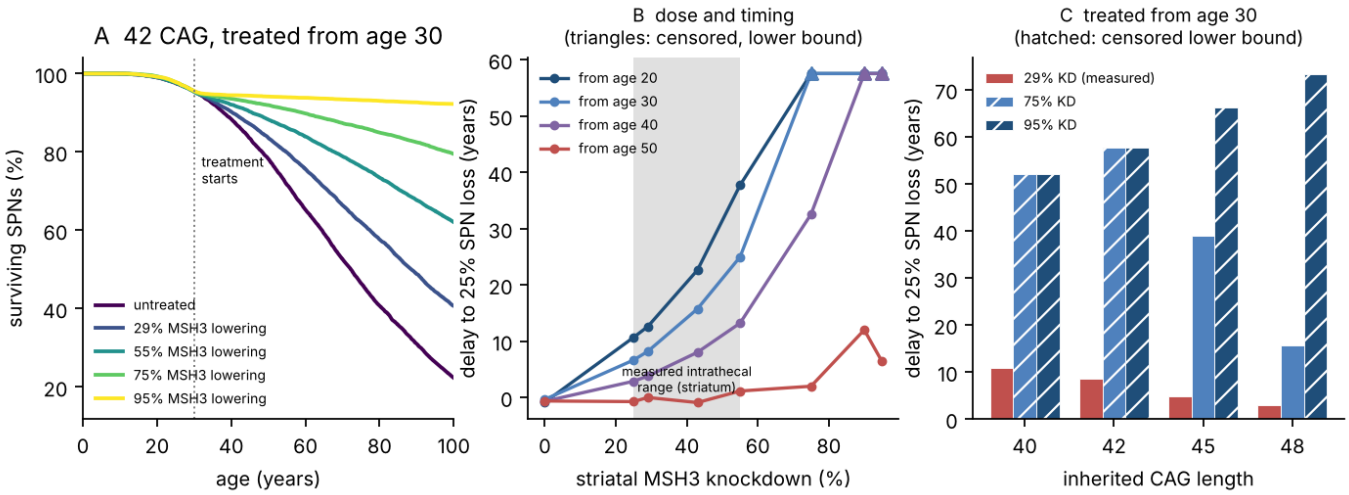


Fig. 11. **In silico trial.** (A) Neuron survival for a 42-repeat carrier treated from age 30. (B) Delay to 25% loss against knockdown and start age; the shaded band is the measured intrathecal range and triangles mark censored lower bounds. (C) Dependence on inherited CAG length.

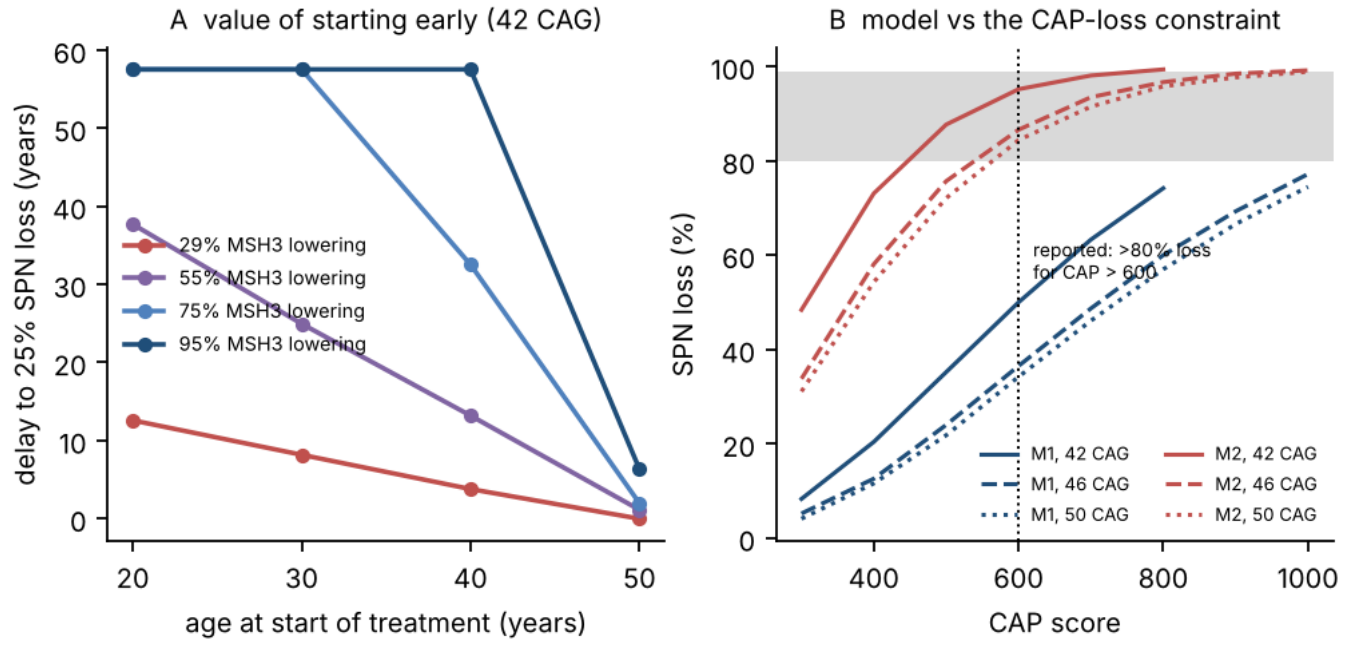


Fig. 12. **Timing, and the outstanding constraint.** (A) Benefit against age at treatment start. (B) Neuron loss against CAP score for both bracketing models, with the reported end-stage range shaded.

TABLE VIII
LIFETIME EFFECT OF REPRESENTATIVE DOSING REGIMENS, AGAINST THE IDEALISED STEP CHANGE, FOR A 42-REPEAT CARRIER TREATED FROM AGE 30.

Regimen	Effective knockdown	Loss at 70	Benefit
Idealised step, 75% lowering	75.0%	13.2%	35.2 points
Six-monthly, six-month half-life	81.8%	10.6%	37.7 points
Six-monthly, three-month half-life	64.0%	16.9%	31.4 points
Twelve-monthly, three-month half-life	39.8%	28.2%	20.1 points
Four-monthly, nine-month half-life	92.6%	7.9%	40.4 points

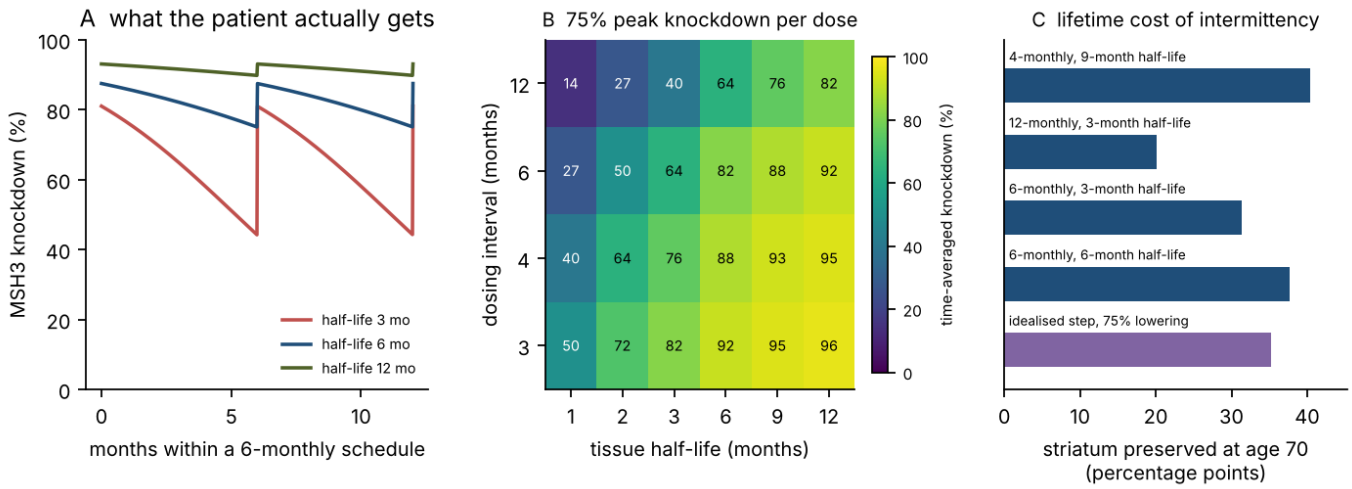


Fig. 13. **What an intermittent schedule actually delivers.** (A) Achieved knockdown through one steady-state cycle of six-monthly dosing, for three tissue half-lives. (B) Time-averaged knockdown across half-life and interval, at 75% peak knockdown per dose. (C) Lifetime benefit for representative schedules against the idealised step change.

TABLE IX
PARTICIPANTS PER ARM FOR 80% POWER AT $\alpha = 0.05$, ENROLLING AT AGE 40 WITH 2-POINT IMAGING MEASUREMENT NOISE.

Arm	2-year trial	3-year trial	5-year trial
29% knockdown (measured achievable)	417 per arm	133 per arm	47 per arm
55% knockdown (best caudate figure)	150 per arm	47 per arm	14 per arm
75% knockdown (aspirational)	86 per arm	28 per arm	9 per arm

Trial duration is the next strongest lever, and it works better than it should. Going from two years to five years cuts the required sample size by roughly nine-fold, far more than the three-fold that a proportional effect would give, because the effect grows faster than linearly with time while the measurement noise does not grow at all. A longer trial is cheaper per unit of statistical power.

The enrolment-age result is less clean and worth flagging as a caution. Enrolling at 50 rather than 40 slightly reduces the required sample size, from 133 to 165 at three years in the main comparison but down to 45 against 47 at five years, because older participants are losing neurons faster and so the absolute change over the trial window is larger. That is a real statistical effect and it is also a trap. A trial enrolling at 50 is easier to power and, per Section III-J, treats people for whom the clinically meaningful endpoint has mostly already passed. Optimising enrolment age for statistical convenience would select against therapeutic benefit, which is precisely the kind of mistake a model like this is useful for catching before it is made.

M. The conclusion survives full uncertainty propagation

Across 160 Latin hypercube samples, with the rate law recalibrated inside every single sample so that calibration uncertainty is propagated rather than frozen at the central fit (Fig. 15; `results/sensitivity_prcc.csv`), the benefit of the measured achievable knockdown of 29% starting at age 30 is 11.4 percentage points of striatum preserved at age 70, with a 95% interval of 7.4 to 18.4. Untreated loss at 70 ranges from 21.0% to 55.6% and treated loss from 11.5% to 40.7% across the sampled space, but the benefit never vanishes in any sample. That is the most important single sentence in this section: the absolute numbers move a lot and the sign of the conclusion does not move at all.

The strongest drivers are the pharmacodynamic order, with a partial rank correlation of +0.73, the expansion step size at -0.71, the contraction rate at -0.68 and the basal transit time at -0.62. It is reassuring that the top driver is the pharmacodynamic order, because that is the quantity most directly constrained by independent data (Section III-B) rather than the one most freely assumed.

The elimination submodel constants, which are the parameters that were simply assumed with no data behind them, all have partial rank correlations below 0.18. The conclusions do not rest on them. The MutSβ footprint has a correlation of essentially zero, for a structural reason worth stating: it enters the rate law multiplicatively through site multiplicity, and calibration absorbs any change in it into the pre-factor A, so it cannot affect the outcome. A parameter taken from a preprint turns out to be immaterial, which is a better position to be in than relying on it.

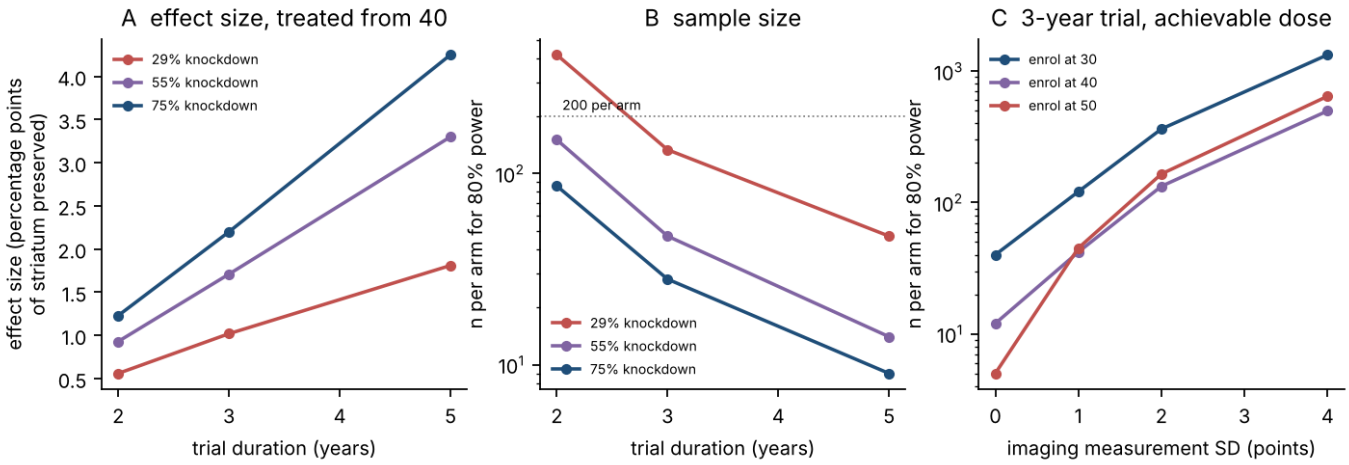


Fig. 14. **Trial feasibility.** (A) Effect size against trial duration for three knockdown levels, treated from age 40. (B) Required sample size per arm for 80% power. (C) Sample size against imaging measurement error for a three-year trial at the achievable dose, for three enrolment ages.

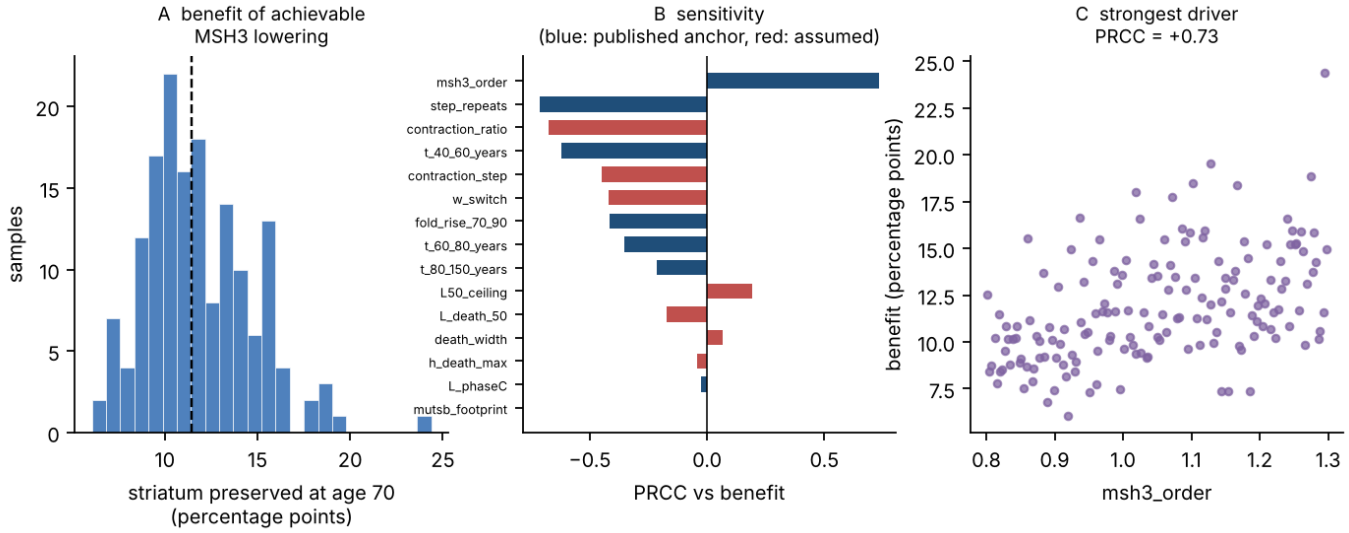


Fig. 15. **Global sensitivity.** (A) Distribution of the predicted benefit across 160 recalibrated samples. (B) Partial rank correlation of each input against benefit; blue inputs are published anchors, red are assumed constants. (C) Response to the strongest driver.

IV. Discussion

A. What the model says

Somatic CAG expansion in this model behaves as a switch rather than a ramp, and the switch sits at roughly eighty repeats, close to the length at which a tract can first host clustered slip-outs that mismatch repair is known to resolve badly. The position of that switch is one of the best-determined quantities in the fit, and the thermodynamic parameter that a more conventional model would have leaned on is not determined at all.

MSH3 lowering acts first-order on the process, so suppression tracks target engagement proportionally. There is no cooperative threshold to clear and no cooperative bonus to collect. Because a neuron that crosses 150 repeats dies within about twenty months, and because reaching 150 takes 97.5% of its life, essentially all therapeutic leverage lies in the pre-threshold decades and essentially none afterwards. That single structural fact explains why accelerating terminal death cannot reconcile the model with end-stage autopsy data, and it explains why treatment started at 50 does so much less than treatment started at 30.

The arithmetic that follows is uncomfortable. The striatal MSH3 lowering that cerebrospinal-fluid-delivered oligonucleotides currently achieve in primates, 25 to 55% from a regimen heavier than clinical dosing and likely worse in a brain four times larger, yields a third to a half of the available suppression and preserves about eleven percentage points of striatum by age 70. That is a genuine disease-modifying effect, and Section III-L says a trial of ordinary size would see it. But it is a small fraction of what the target can deliver, since 75%

knockdown pushes the same cohort past the end of a simulated lifespan.

B. An engineering specification, written in numbers

The point of building a model like this is that it turns a biological ambition into figures a formulation or device team can work against.

1) **Target engagement of at least 70% reduction in MSH3 protein in putaminal projection neurons.** Not in cerebrospinal fluid, not in cortex, and not in bulk striatal tissue. Bulk measurement underestimates what is needed, because projection neurons are the cells that expand and they are a minority of the tissue.
2) **Begin before age 30**, and earlier for carriers above 45 repeats. Benefit roughly halves per decade of delay, and by 50 the delay to 25% loss is gone entirely.
3) **Required tissue exposure of about 25 nmol per gram**, which is about 7 mg inside a human striatum.
4) **Dose at an interval no longer than the tissue half-life.** Annual dosing with a three-month half-life throws away 15 of the 35 available percentage points of benefit. This costs nothing to fix and is the cheapest improvement available anywhere in this analysis.
5) **A systemic transcytosis route needs an 18 to 36-fold improvement** in parenchymal delivery over the best measured transferrin-receptor design. Affinity and valency optimisation, to monovalent binding at K_D near 135 nM, supplies about two-fold of that. Endosomal escape is the dominant remaining term and the most promising place to engineer.
6) **The trial endpoint must be striatal MSH3 protein**, or a validated surrogate such as a somatic expansion index in a reachable compartment. Cerebrospinal fluid drug concentration is not sufficient, because the entire difficulty is the step from cerebrospinal fluid to putamen, and a concentration measured upstream of the hard step says nothing about whether the hard step happened.
7) **Invest in imaging reproducibility alongside target engagement.** A reduction in endpoint measurement error from 4 to 2 percentage points cuts the required sample size nearly four-fold, which is a larger effect than doubling target engagement.

C. Predictions that could falsify this

- Clinical motor onset should correspond to 18 to 31% projection-neuron loss, rising with inherited CAG length. Much more than that and the dwell-time-faithful model is wrong.
- The relationship between CAP score and neuron loss should follow the trajectory in Fig. 12B. A well-powered post-mortem series spread across the CAP range would discriminate the two bracketing models immediately, and would resolve the one genuine inconsistency in this work.
- Because the rate law is first-order in MSH3, partial MSH3 lowering should suppress somatic expansion in direct proportion in human striatal neurons, with no cooperative threshold. This is directly testable in patient-derived striatal cultures, where antisense-mediated MSH3 suppression has already been shown to reduce expansion [11]; what has not been done is a dose titration fine enough to distinguish first-order from cooperative behaviour.
- Expansion rate should rise sharply rather than smoothly as tracts pass eighty repeats, and the position of that rise should shift with anything that alters slip-out spacing. CAA interruption is the obvious test case, and Section III-G gives the predicted size of the shift in years.
- Indirect-pathway depletion should be about six percentage points at half neuron loss, should scale with total loss rather than with age or genotype, and should not exceed about ten points at any stage. A reported depletion much larger than that cannot be explained by a 15% rate difference.

D. Where this sits in the field

The demonstration that FAN1 opposes MutLγ directly [5] fits into this framework without strain, as a modifier of the escape probability R, which is precisely the parameter the model identifies as carrying the length dependence. That gives a mechanistic account of why FAN1 variants are among the strongest onset modifiers known, and Section III-G quantifies it: FAN1 is the single strongest lever in the model and also the least tractable pharmacologically.

The model also implies that huntingtin-lowering and MSH3-lowering are not competitors but complements that act on different parts of the same timeline. MSH3 lowering extends the pre-threshold phase, during which a neuron is alive and not yet carrying a harmful tract. Huntingtin lowering can only address toxicity once a cell has crossed the threshold. On this reading a combination should be more than additive, since one arm delays arrival at the threshold and the other reduces the consequence of arriving, and nothing in this model makes them interfere.

Finally, there is a methodological point worth making. Every quantitative claim in this paper came from four published numbers fitted to a four-parameter law, and everything else, including five held-out validations, three modifier predictions, a subtype prediction and a trial power calculation, followed without further fitting. The surprise is not that the fit worked, since four parameters against four targets should fit. The surprise is how much the fitted structure then predicted that it was never shown.

V. Limitations

1) **No primary data.** Every calibration target is a published summary statistic, and several of them are themselves model-derived. The transit times in [1] come from a simulation rather than from longitudinal measurement of single cells, so this model inherits the assumptions of that one.
2) **The functional form of the rate law is assumed.** Each factor in $\lambda(L)$ is sourced, but writing them as

a product is not: it assumes the steps are independent and sequential. The fit cannot test that assumption, because a different combination rule with its own four free parameters would very likely also reproduce four anchors. What the data constrain is the shape of the length dependence, not the algebra that produces it.

3) **A key biochemical source is a preprint.** The MutSβ footprint and stoichiometry [3] come from a bioRxiv preprint that has not been peer reviewed. The sensitivity analysis shows the footprint is immaterial, for the structural reason given in Section III-M, but the qualitative tandem-binding picture is not independently confirmed here.
4) **The discrepancy with end-stage autopsy data is unresolved**, not explained away. Two independent published constraints cannot both be satisfied by this model structure. The resolution adopted, that "CAP above 600" describes a late-stage population rather than a threshold, is reasonable and is the least convenient reading for the author's own argument, but it is not demonstrated.
5) **Species scaling is crude.** The macaque-to-human penetration correction uses cube-root geometric scaling and an assumed putaminal depth. The direction is robust, the magnitude is not, and a real answer would need a diffusion model on actual human anatomy.
6) **The pharmacodynamic link crosses species and timescales.** Proportionality between MSH3 protein and expansion suppression was measured in mouse striatum over three months and is assumed here to hold in human projection neurons over decades.
7) **Nanocarrier pharmacokinetics collapse into a single efficiency term.** Plasma clearance, protein corona formation, opsonisation, hepatic first pass, receptor saturation at therapeutic doses and chronic-dosing tolerability are all absent. Every one of them would make the systemic route harder rather than easier, so the stated dose requirement should be read as a lower bound.
8) **Endosomal escape at 2% is assumed**, and it scales the systemic dose requirement linearly. It is the least constrained number in the delivery analysis, which is why it is swept rather than asserted in Fig. 10B.
9) **Neurons are independent** except in the explicit coupling test of Section III-D, and non-neuronal contributions are omitted entirely. Astrocytic glutamate handling, microglial activation and loss of corticostriatal input are all part of the real disease and none of them are here.
10) **Treatment is a step change in the main trial analysis.** Section III-K removes this for the first-order case, exactly rather than approximately, but the correction relies on first-order pharmacodynamics and would not transfer to a cooperative mechanism.
11) **Safety is not modelled at all.** Lifelong suppression of a mismatch-repair protein beginning in the third decade raises mutational and oncological questions this work does not address. MSH6 protein rose to 142% of control at the highest dose in the mouse study [4], suggesting compensatory shifts within the repair machinery whose long-term consequences are unknown. A specification that says "start before 30 and continue for life" is a specification with an unquantified risk attached, and no amount of modelling of efficacy substitutes for measuring that risk.
12) **No sequence-level design was attempted.** An oligonucleotide design module was planned and dropped, because the sequence databases needed to do it rigorously were unreachable from this environment and a sequence written from memory would have been unverifiable. Section VI records what was blocked.
13) **The trial power calculation assumes a normal endpoint and no dropout.** Real trials lose participants, and a right-censored survival endpoint would need a different calculation. The sample sizes given should be read as a floor.

VI. Reproducibility

Run in numerical order from `code/`. Total runtime is about forty minutes on a single core, most of it in the sensitivity analysis, which recalibrates the model 160 times. Every random seed is fixed, so the figures and tables reproduce exactly. Every number quoted in this paper is written to `results/` as CSV or JSON by the script that produced it, and no number was transcribed by hand.

Dependencies are Python 3.12 with NumPy, SciPy and Matplotlib. The document build additionally needs pandoc and LibreOffice.

What could not be done from this environment. The NCBI Entrez and Crossref programmatic interfaces were blocked by the network policy, PubMed Central required browser verification, and several publisher sites were unreachable or rate-limited. The practical consequences were two. First, the planned oligonucleotide sequence-design module was dropped rather than attempted from memory. Second, where a retrieved page did not expose a volume or page range, the reference below gives a DOI instead of a guessed citation. No reference appears here that was not opened and read.

References

[1] R. E. Handsaker, S. Kashin, N. M. Reed, *et al.*, "Long somatic DNA-repeat expansion drives neurodegeneration in Huntington's disease," *Cell*, 2025, PMID 39824182.

[2] "The breaking point where repeat expansion triggers neuronal collapse in Huntington's disease," *Cell Genomics*, vol. 5, no. 4, 2025. Commentary on [1].

[3] J. Li, H. Wang, and W. Yang, "Tandem MutSβ binding to long extruded DNA trinucleotide repeats underpins pathogenic expansions," *bioRxiv* 2023.12.12.571350, 2023, doi:10.1101/2023.12.12.571350. Preprint, not peer reviewed.

[4] "Dose-dependent reduction of somatic expansions but not Htt aggregates by di-valent siRNA-mediated silencing of MSH3 in HdhQ111 mice," *Scientific Reports*, 2024, doi:10.1038/s41598-024-52667-3.

[5] I. Senoussi, V. Mengoli, A. Cerana, *et al.*, "Mechanism of trinucleotide repeat expansion by MutSβ and MutLγ and contraction by FAN1," *Nature Communications*, vol. 16, p. 9445, 2025, doi:10.1038/s41467-025-64485-w.

TABLE X
CONTENTS OF THE CODE REPOSITORY. SCRIPTS RUN IN NUMERICAL ORDER; EACH WRITES ITS OWN RESULT TABLES.

Path	Contents
`code/hdmodel.py`	core model: rate law, calibration, Gillespie and tau-leaping solvers
`code/01_calibrate.py`	calibration, identifiability, MutSbeta stoichiometry gives Fig 1-2
`code/02_population.py`	population simulation, held-out validation gives Fig 3
`code/03_coupling.py`	solver verification, non-cell-autonomous test gives Fig 4
`code/04_reconcile.py`	reconciliation map, bracketing models gives Fig 5
`code/05_delivery.py`	exposure-response, CSF route, nanocarrier design space gives Fig 6-7
`code/06_trial.py`	in silico trial gives Fig 8-9
`code/07_sensitivity.py`	Latin hypercube with recalibration, PRCC gives Fig 10
`code/08_translation.py`	dose translation to mg/kg
`code/10_profile.py`	parameter profiles and feasible ranges gives Fig 11
`code/11_subtypes.py`	direct and indirect pathway prediction gives Fig 12
`code/12_modifiers.py`	genetic modifiers as parameter perturbations gives Fig 13
`code/13_dosing.py`	intermittent dosing and time-averaged knockdown gives Fig 14
`code/14_trial_power.py`	sample size and power for a real trial gives Fig 15
`code/09_build_documents.py`	builds the Word and PDF versions of this paper

[6] G. B. Panigrahi *et al.*, "Isolated short CTG/CAG DNA slip-outs are repaired efficiently by hMutSβ, but clustered slip-outs are poorly repaired," *Proceedings of the National Academy of Sciences USA*, 2010, doi:10.1073/pnas.0909087107.

[7] J. A. Frei, J. E. Gentile, Y. Lian, M. A. Mortberg, J. Capitanio, P. Jafar-nejad, S. M. Vallabh, H. T. Zhao, and E. V. Minikel, "Cell type distribution of intrathecal antisense oligonucleotide activity in deep brain regions of non-human primates," *Nucleic Acid Therapeutics*, 2025, doi:10.1177/21593337251371594.

[8] K. B. Johnsen, M. Bak, P. J. Kempen, F. Melander, A. Burkhart, M. S. Thomsen, M. S. Nielsen, T. Moos, and T. L. Andresen, "Antibody affinity and valency impact brain uptake of transferrin receptor-targeted gold nanoparticles," *Theranostics*, vol. 8, no. 12, pp. 3416–3436, 2018, doi:10.7150/thno.25228.

[9] D. O'Reilly *et al.*, "Di-valent siRNA-mediated silencing of MSH3 blocks somatic repeat expansion in mouse models of Huntington's disease," *Molecular Therapy*, 2023.

[10] M. Orth and C. Schwenke, "Age-at-onset in Huntington disease," *PLOS Currents Huntington Disease*, ed. 1, Jul. 2011, doi:10.1371/currents.RRN1258.

[11] "Antisense oligonucleotide-mediated MSH3 suppression reduces somatic CAG repeat expansion in Huntington's disease iPSC-derived striatal neurons," *Science Translational Medicine*, 2025, doi:10.1126/scitranslmed.adn4600.

[12] Enroll-HD, "Special characteristics of HD data," citing J. H. Warner *et al.*, "Standardizing the CAP score in Huntington's disease." [Online]. Available: https://enroll-hd.org/for-researchers/analyzing-data/